

Ovotech Medical Coding

Features Advantages and Benefits

Use this guide after your demonstration to assess how Medical Coding could fit your practice. Share it with clinical, operational, information governance, finance and procurement colleagues, then agree the workflow, evidence and measures needed for an evaluation.



Illustrative scene with fictional information.

Ovotech is building the AI Operating Layer for Healthcare. Medical Coding is its service for clinical correspondence, combining structured information, source-backed SNOMED CT suggestions, review and controlled patient record updates. SNOMED CT is a standardised clinical terminology.

Inside this guide: workflow capabilities, potential organisational benefits, baseline measures and the questions to resolve before implementation.

Correspondence and clinical review

Feature = capability. Advantage = the change in the workflow. Potential benefit = the organisational value to evaluate.

Structured correspondence extraction

Advantage: Relevant details appear in a consistent format beside the original correspondence, reducing repeated searching and re-keying.

Potential benefit: The practice can test whether reviewers release time for other work while retaining a clear source to check.

Measure: Average handling minutes per document, including corrections and rework.

Source backed SNOMED CT suggestions

Advantage: Suggested concepts remain linked to supporting text, so reviewers can assess each suggestion in context.

Potential benefit: Teams can evaluate review capacity and decision consistency without removing professional judgement.

Measure: Review time, decision outcomes and items returned for further review.

Visible uncertainty and exceptions

Advantage: Missing information, unmatched documents and exceptions remain identifiable for investigation and prioritisation.

Potential benefit: The practice can direct attention to unresolved work and reduce the chance of delays going unnoticed.

Measure: Exception age, cause, unresolved volume and time to resolution.

Medical Coder and Clinical Reviewer workspace

Advantage: The same clinical role uses one connected queue for source documents, extracted fields, coding decisions and status.

Potential benefit: Reviewers gain clearer daily priorities, while the practice can assess throughput and the distribution of work.

Measure: Open assignments, completed documents per reviewer and queue age.

Completion and practice oversight

Feature = capability. Advantage = the change in the workflow. Potential benefit = the organisational value to evaluate.

Controlled patient record updates

Advantage: Accepted selections follow the configured approval route. Update status and exceptions show what completed and what needs attention.

Potential benefit: Teams can evaluate fewer manual hand-offs and clearer responsibility for completing the patient record.

Measure: Successful updates, update exceptions and time from approval to completion.

Practice operational overview

Advantage: Volume, backlog, turnaround, coder workload and exceptions are brought into an operational view.

Potential benefit: Managers can prioritise work and discuss staffing or process changes using the same operational picture.

Measure: Average turnaround, backlog trend, processing volume and workload distribution.

Role based permissions and traceability

Advantage: Clinical access and operational oversight follow explicit permissions, with relevant actions linked through the intended workflow history.

Potential benefit: The practice can maintain clearer accountability and investigate how decisions and outcomes were reached.

Measure: Access reviews, decision records and time spent investigating exceptions.

Practice specific value baselines

Advantage: Observed handling time and turnaround are compared with the practice's current process and relevant staff cost.

Potential benefit: Clinical and operational colleagues can build an internal case using their own evidence and measured capacity changes.

Measure: Time difference, estimated hours released, staff-time equivalent and turnaround difference.

Evaluating the impact

Agree the starting point before testing. Compare the same document mix and period, and include exception handling and rework in the measurement. Processing duration, staff handling effort and elapsed turnaround are different measures.

Baseline	What to record	How to compare
Monthly correspondence volume	Documents handled during a representative month.	Compare the same document types and workload period.
Current manual handling time	Average minutes of staff effort per document.	Include reading, coding, corrections and rework.
Current average turnaround time	Elapsed hours from receipt to the completed outcome.	Use the same start and finish definition in both workflows.
Staff hourly cost	An agreed cost for the staff time being compared.	Use the practice's own finance assumptions.
Backlog and exceptions	Open, overdue and unresolved work.	Check whether time has moved into another stage.

Calculating the difference

Estimated hours released: Monthly documents × (current manual minutes minus observed workflow minutes) ÷ 60.

Staff time equivalent: Estimated hours released × the agreed hourly staff cost.

Turnaround difference: Current average turnaround minus observed average turnaround, using the same units.

A negative result means additional effort or slower turnaround and should remain visible. Staff-time equivalent is a capacity estimate, not a guaranteed cash saving. Include implementation, licence, support and training costs separately when assessing the overall business case.

A practical evaluation plan

- Agree a representative sample, evaluation period and named owners.
- Record the baseline and define success criteria before the evaluation.
- Review the results with clinical and operational colleagues, including exceptions.
- Decide whether the evidence supports proceeding, adjusting the workflow or investigating further.

Governance and next steps

The Medical Coder and Clinical Reviewer are the same clinical role. Practice administrators oversee operations; this does not automatically grant access to clinical documents or coding decisions. A person can hold both permissions where authorised.

Review and patient record updates

The manual route includes review and authorised approval before the patient record is updated. Where supported, practice policy may allow policy-based approval and automated updates for eligible documents. Confirm the configured route, exception handling and clinical-system compatibility during evaluation.

Ovotech document history covers correspondence processed through Ovotech. Do not assume that it represents the complete historical clinical record.

Assurance status

Framework or certification	Status
DTAC	Assessed
DSPT	Standards met
ISO 27001	Certified
Cyber Essentials	Certified

Request the relevant assurance documentation, certification scope and deployment-specific arrangements for your internal review.

Questions for your internal discussion

Clinical review: Which correspondence types will be evaluated, who reviews them and how are uncertain suggestions resolved?

Practice operations: Who owns the queue, exceptions and review of workload and turnaround?

Information governance and security: What access, hosting, retention, contractual and assurance evidence is required?

Finance and procurement: Which baseline, evaluation period and success criteria will support the decision?

Implementation: Which clinical system, permissions, configuration, training and acceptance checks are needed?

Use fictional information in demonstrations. Agree any use of real correspondence through the appropriate governance and implementation arrangements.

Product workflow and client resources: [ovotech-healthcare-platform.vercel.app](#)